

HUMANITARIAN

UPDATE

HIGHLIGHTS

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March & April 2026

Conflict-affected IDPs in the eastern region. Photo: Coordination of Humanitarian Assistance

CROSS-BORDER HOSTILITIES CONTINUE TO DRIVE HUMANITARIAN NEEDS IN AFGHANISTAN

Cross-border hostilities between Afghanistan and Pakistan since October 2025 have caused significant humanitarian consequences, including civilian casualties, displacement and disruptions to essential services. Although a ceasefire reached in October 2025 temporarily reduced the intensity of hostilities, violence has continued intermittently in border provinces.

Between January and March 2026, cross-border armed violence between Afghanistan's de facto security forces and the Pakistani military resulted in 769 civilian casualties, according to the United Nations Assistance Mission in Afghanistan (UNAMA). This includes 372 people killed and 397 injured, among them 72 women, 554 men, 48 girls and 95 boys.

UNAMA reports that the majority of incidents occurred in February and March, largely driven by airstrikes following Pakistan's announcement

of Operation Ghazab lil-Haq on 26 February. Airstrikes accounted for approximately 64 per cent of total civilian casualties, while the remainder resulted from indirect cross-border fire and isolated incidents, including the targeted killing of a humanitarian worker.¹

Displacement and damage to civilian infrastructure

Humanitarian partners estimate that more than 100,000 people have been displaced across Khost, Kunar, Nangarhar, Nuristan, Paktia, and Paktika provinces since the escalation of hostilities. While displacement has largely been temporary, affected populations continue to experience significant humanitarian needs.

Assessments indicate that airstrikes or clashes linked to cross-border violence have been reported in 11 provinces. In addition, nearly 1,000 homes have been damaged or

destroyed. Civilian infrastructure, including schools, health facilities and water systems, has also been damaged or destroyed.

Approximately 19 health facilities have been suspended, closed or reduced to partial operations, limiting access to health care for an estimated 78,000 people. In the education sector, more than 13,000 students in Kunar and Nangarhar provinces have experienced disruptions.²

Access constraints and operational challenges

Humanitarian access has been constrained in several affected areas due to insecurity, shelling, and road closures. In Kamdesh and Bargmatal districts of Nuristan Province, access to nearly 100,000 people was restricted for almost two months, limiting the delivery of essential services. The main supply

1. UNAMA HUMAN RIGHTS MONITORING AND REPORTING | UNAMA

2. Afghanistan Situation Update #4: Humanitarian Impact of Afghanistan-Pakistan Military Escalation (28 April 2026) - Afghanistan | ReliefWeb

route between Nari District in Kunar Province and Kamdesh District in Nuristan Province was closed due to ongoing hostilities, causing shortages of food, medicine, and fuel, and disrupting health referral pathways and humanitarian operations.

On 13 April, the road was reopened following engagement by community elders from both sides and humanitarian actors. Since then, humanitarian organizations have resumed the delivery of food, medical supplies and other essential assistance to affected communities.

Humanitarian needs and ongoing response

Despite intermittent improvements and ongoing humanitarian response efforts, humanitarian needs remain severe across the affected provinces. Needs assessments conducted in affected areas have identified critical gaps in food security, emergency shelter, health services, nutrition support, water, sanitation and hygiene and multi-purpose cash

assistance.

The World Food Programme estimates that approximately 160,000 people in affected areas are facing heightened food insecurity. Nutrition partners have also raised concerns regarding acute malnutrition among children under five, as well as pregnant and breastfeeding women. Protection risks remain high in conflict-affected areas, particularly for women and children exposed to displacement, shelling, and explosive remnants of war.

While the intensity of hostilities has fluctuated in recent weeks, the humanitarian situation remains fragile. Continued insecurity, restricted access and funding shortfalls are likely to sustain humanitarian needs in the coming months.

Humanitarian partners emphasize the importance of sustained humanitarian access, the protection of civilians, and adequate funding to ensure the continuity of life-saving assistance.

FLOODS IN AFGHANISTAN: WIDESPREAD DESTRUCTION AMID COMPOUNDING CLIMATE RISKS

Between late March and April, heavy rainfall triggered widespread flash flooding across Afghanistan, affecting 31 of 34 provinces, 165 districts, and 546 villages, approximately one-third of the country. The floods caused significant loss of life, extensive damage to homes and infrastructure, and major disruption to livelihoods, leaving thousands of families in urgent need of humanitarian assistance.

Scale of impact and humanitarian needs

An estimated 31,000 people (9,984 families) were affected by the floods across Afghanistan. At least 55 people were killed and 148 injured. In total, 7,751 houses were damaged and 1,731 destroyed, forcing many families into temporary shelter arrangements or unsafe living conditions.³



The eastern region, particularly Nangarhar Province, was among the most affected, followed by southern and western regions. Rural communities were among the hardest hit, with widespread damage to agricultural land, irrigation systems, and rural infrastructure further weakening already fragile livelihoods and food security.

Beyond immediate shelter needs, the floods have significantly deepened existing vulnerabilities. Many households have lost both their homes and primary sources of income, particularly in rural areas dependent on agriculture and seasonal labour.

Displacement patterns remain fluid. While some families have returned to partially damaged homes due to a lack of alternatives, others remain in temporary shelters. Access to safe water, sanitation, and basic services remains limited in several affected districts, increasing health and protection risks.

Humanitarian response

Humanitarian partners, in coordination with local authorities, have scaled up emergency response efforts across affected areas. As of May, 37,845 people have received assistance (including 22,821 people with food assistance, 37,536 with non-food items, 16,151 with shelter support, 32,473 with WASH assistance and 21,232 with cash assistance).

Response efforts are ongoing, but challenges persist

due to damaged infrastructure, access constraints, and limited resources in some of the hardest-hit areas. Funding constraints are also contributing to critical gaps in assistance, particularly in hard-to-reach and severely affected communities where needs remain unmet.

Multiple shocks, growing needs

The recent floods are part of a broader pattern of climate-related shocks affecting communities across Afghanistan. Environmental pressures, fragile infrastructure and climate change continue to increase exposure to disasters and deepen existing vulnerabilities.

At the same time, prolonged drought conditions continue to affect large parts of the country. Afghanistan is facing its sixth consecutive year of drought-like conditions, with continued below-normal precipitation and higher temperatures expected into 2026.

These overlapping shocks are compounding humanitarian needs, particularly in rural areas where households have limited coping capacities and depend heavily on agriculture and seasonal labour.

Sustained humanitarian support remains critical to address immediate needs and help strengthen the resilience of communities increasingly exposed to recurring climate shocks.

A SMALL CLINIC IS A LIFELINE FOR REMOTE COMMUNITIES IN KABUL PROVINCE, BUT ITS FUTURE IS UNCERTAIN

For families in the Paghman district of Kabul Province, even the most basic health care is often out of reach, requiring long walks across difficult terrain, journeys that can delay critical, sometimes lifesaving, treatment for women and children.

In these isolated communities, a small clinic supported by Johanniter International Assistance (JUH), with funding from the Afghanistan Humanitarian Fund (AHF) and co-financing from the European Union through the European Civil Protection and Humanitarian Aid Operations (ECHO), has become a lifeline.

The project focuses on underserved communities where humanitarian assistance is limited, and where internally displaced people, returnees and host communities face persistent barriers to health care. Through support to health facilities and outreach services, it provides primary health care, maternal and child health services, nutrition treatment,

immunization, mental health and psychosocial support, treatment for common illnesses and health education.

For many residents, the clinic is the only accessible source of health care. "This is the only clinic around here," said Amina Noorzai, a nutrition counsellor who has worked at the facility for the past eight months with JUH. "Before this, people had to travel nearly 10 kilometers to reach another health center. Most families could not afford transportation and many women simply stayed home when they became sick."

Each morning, the clinic's waiting area fills early. Mothers sit closely together holding infants as health workers move between patients, checking vital signs, providing counselling, and distributing medicines. Nearly 80 per cent of patients are women and children.



Health facilities in Paghman District, Kabul, supported by the Afghanistan Humanitarian Fund and co-financed by the EU/ECHO. Photo: OCHA

For Amina, the work is deeply personal. One case still haunts her. “It was one of my first weeks here,” she said quietly. “A seven-month-old child came for growth monitoring and counselling, suffering from severe acute malnutrition and dehydration. Later, I learned the child had died.” The condition, she explained, was treatable — but the family arrived too late. “They did not know it could be treated,” she said. “If they had come earlier, we might have saved her.”

Health workers say such cases are common in remote communities, where poverty, low literacy and limited health awareness often prevent families from seeking timely care. Many people have never received basic information about nutrition, hygiene or early signs of illness.

As part of the project, health-care workers conduct awareness sessions to help families recognize symptoms early and understand when to seek treatment. “We try to teach people that diseases can be cured,” Amina said. “They should not wait until it becomes an emergency.”

The clinic also provides mental health and psychosocial support, services that are rarely available in remote Afghan communities despite widespread trauma linked to poverty, displacement and years of crisis. “Many women come here carrying emotional pain that nobody talks about,” she said. “Mental health struggles are often hidden, but it is very important. We tell them treatment is available and they can come here for help.”

Sustained humanitarian support remains urgently needed. In Paghman district alone, the closure of this Basic Health Center in 2026 will leave an estimated 12,000 people without access to essential health-care services.

Without continued donor funding, vulnerable communities risk once again losing access to even the most basic medical care a gap that, in remote and hard-to-reach areas, can quickly turn treatable illnesses into life-threatening emergencies.

“We hope this project continues,” Amina said. “These communities really need health-care support. Even basic services can save lives.”

MONTHS AFTER THE EARTHQUAKE, FAMILIES IN KUNAR CONTINUE STILL STRUGGLING TO RECOVER

Several months after the powerful earthquake that struck eastern Afghanistan in August 2025, families across Kunar Province are still struggling to recover from the disaster. While emergency assistance

provided critical support in the immediate aftermath, many households remain without adequate shelter, stable livelihoods and long-term recovery support.⁴

4. This story was produced with the support of OCHA Afghanistan colleagues at the sub-national level, Said Alam Khan.

For 35-year-old Abdul Majeed Khan from Gorbaz village in Mazar Dara, Nurgal district, the earthquake continues to shape every aspect of daily life.

"It was around midnight when the earthquake happened," Abdul Majeed recalls. "We lived in a three-storey house. When we realized what was happening, we were all buried under rubble and dust."



Earthquake-affected Abdul Majeed Khan, 35, in Nurgal District. Photo: OCHA

Within moments, the family home collapsed, causing devastating losses. Abdul Majeed lost his father and three of his children. He, his wife and his sister survived with serious injuries. Months later, Abdul Majeed still walks with crutches. "For the first three months, I could not walk at all," he says.

Rescue teams reached the area the following morning and transferred injured family members by air to Jalalabad for emergency treatment. Abdul Majeed spent nearly four months in hospital and still requires follow-up medical care. Beyond the physical injuries, the earthquake caused severe damage to the family's living conditions and financial security. The family's five-room house, built over many years, was

completely destroyed. "Our house was completely destroyed and nothing was left," he says.

The disaster also wiped out the family's primary source of livelihood. Four cows that supported household income died when the house collapsed. "I had four cows, and all of them died," he says. Abdul Majeed's sister also sustained injuries that left her permanently disabled, adding further challenges to a family already coping with loss and displacement.

Today, the family remains in temporary shelter under arduous conditions. Without permanent housing, livestock or a reliable source of income, recovery remains a significant challenge.

Following the disaster, the family received humanitarian assistance including emergency shelter, food assistance, cash support and winter clothing. While this support provided essential relief during the immediate response phase, Abdul Majeed says families now require support that can help them rebuild their lives. "Families need long-term support to stand on their own feet," he says.

He also highlights the importance of livelihood opportunities, including support for women, to help families restore income and strengthen their resilience.

Across Kunar Province, many earthquake-affected families continue to face similar challenges. Although emergency assistance helped address urgent needs in the immediate aftermath, housing support, livelihood recovery and sustained assistance remain urgent priorities for communities still struggling to recover.

For survivors like Abdul Majeed, rebuilding means more than replacing what was lost; it means creating the conditions for families to recover and move forward.

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